# Dacomitinib monotherapy

**Which treatment this guide covers** NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Dacomitinib alone · daily oral treatment

**Active medicines:** dacomitinib

**Emetogenic risk:** Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.
- Dacomitinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.
- Acid-suppressing medicines can reduce dacomitinib exposure. Proton pump inhibitors should be avoided if possible; ask pharmacy about alternatives. NCCP and current product information give different timing before H2 blockers, so obtain a written individual schedule. CYP2D6 medicines also need pharmacy review. Do not change medicines yourself.
- Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 17 days after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____



Sources checked 2026-10-29 · SACTCheck v0.84.25 · Regimen-level drafts; prescriber and oncology-pharmacy verification required